3.4 Chronic Obstructive Pulmonary Disease (COPD)*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "I cannot climb stairs without stopping." **Objective** • Barrel chest • Pursed-lip breathing • SpO_2 88%, $PaCO_2$ 52 mmHg	**ACTUAL** Impaired gas exchange related to alveolar destruction and air trapping as evidenced by hypoxemia, hypercapnia and dyspnea.	• SpO_2 88–92%, or the prescribed target, within 24 hours.	• Give controlled oxygen to the prescribed target; high concentrations are avoided. • Follow ABG, SpO_2 and consciousness. • Sit him upright or leaning forward. • Teach pursed-lip and diaphragmatic breathing. • Give bronchodilators and corticosteroids as prescribed.	• Goal reached when SpO_2 is on target and he is alert. • Drowsiness is reported, as it may mean CO_2 retention.
Subjective • "I get tired when I wash or dress." **Objective** • Breathless on minimal effort • HR rises by 25/min with activity	**ACTUAL** Decreased activity tolerance related to imbalance between oxygen supply and demand as evidenced by exertional dyspnea and fatigue.	• Completes ADL with rests, keeping SpO_2 on target.	• Build rest periods between activities. • Teach energy-saving methods, such as sitting to wash and keeping items close. • Encourage pulmonary rehabilitation. • Check SpO_2 during activity.	• Met if ADL are done with tolerable breathlessness.
Subjective • Smoker, 40 pack-years **Objective** • Thick secretions • On corticosteroids • Frequent admissions	**RISK** Risk for infection as evidenced by risk factors: chronic lung disease, retained secretions and corticosteroid therapy.	• No new chest infection.	• Encourage influenza and pneumococcal vaccination. • Teach the early signs: change in sputum color or amount, fever. • Support him to stop smoking. • Encourage fluids and airway clearance.	• Achieved if no infection develops.

*References for this care plan: [17], [26], [35], [37], [106]. Full entries are listed in the References section of the CPMN book.